



Test Results
Telephone: (604) 431-7206
Toll Free: 1-800-431-7206
Toll Free Fax: 1-877-412-4440

Facsimile Transmittal

To: DR ARIANE QUELLET-DECOSTE

From: Client Information Centre

To Fax: 18776610185

Fax: (604) 412-4445

Phone:

Phone: (604) 431-7206

Comments:

This Fax includes the following reports: 409012011

Performing Location Legend : (for more information go to www.lifelabs.com)

BRL – BURNABY KRL – KAMLOOPS PRL – PRINCE GEORGE VRL - VICTORIA
CCB – CAM COADY NRL – KIMBERLEY TRL - TERRACE

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PATIENT'S NAME	LAB NUMBER	CHART NUMBER	INTERNAL USE ONLY	PG
			7-SUM Q1703	1
			REQ:	
DR ARIANE QUELLET-DECOSTE CORAL HEALTH INC 2900-550 BURRARD ST VANCOUVER BC V6C 0A3			 Test Results: Telephone: 604-431-7206 Toll Free: 1-800-431-7206	

DATE AND TIME COLLECTED	DATE RECEIVED	LAB		
12 JAN 2026 07:21	12 JAN 2026 07:21	458 PNT		
TEST	OUT OF RANGE	WITHIN RANGE	REFERENCE INTERVALS	LIFELABS TEST LOCATION

HEMATOLOGY PANEL

WBC	6.3	4.0--10.0	10 ⁹ /L	VRL
RBC	4.39	3.50--5.00	10 ¹² /L	VRL
Hemoglobin	126	115--155	g/L	VRL
Hematocrit	0.40	0.35--0.45	L/L	VRL
MCV	90	82--98	fL	VRL
MCH	28.7	27.5--33.5	pg	VRL
MCHC	318	300--370	g/L	VRL
RDW	11.9	11.5--14.5	%	VRL
Platelet Count	248	150--400	10 ⁹ /L	VRL

WBC DIFFERENTIAL

Neutrophils	2.8	2.0--7.5	10 ⁹ /L	VRL
Lymphocytes	2.7	1.0--4.0	10 ⁹ /L	VRL
Monocytes	0.4	0.1--0.8	10 ⁹ /L	VRL
Eosinophils	0.3	0.0--0.7	10 ⁹ /L	VRL
Basophils	0.1	0.0--0.2	10 ⁹ /L	VRL
Immature Gran(IG)	0.0	0.0--0.1	10 ⁹ /L	VRL

Glucose fasting	5.1	3.3--5.5	mmol/L	VRL
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HEMOGLOBIN A1C	5.3	4.5--5.9	%	VRL
Therapeutic target for most adults with type 1 or type 2 diabetes is <=7.0%. In the frail elderly and patients who are prone to hypoglycemia, target is <=8.5%. A1c >=6.5% meets the criterion for type 2 diabetes mellitus in adults. See 2018 Diabetes Canada guidelines.				

Creatinine	55	45--90	umol/L	VRL
eGFR	117	>=60		VRL
Units for eGFR are mL/min/1.73sq.m (SEE NEXT PAGE)				

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eGFR

VRL

Kidney function estimate based on assumption of a stable serum creatinine concentration: diet, drugs, pregnancy, clinical state and muscle mass can affect accuracy of the estimate. Urinary ACR may assist interpretation.
See <https://www2.gov.bc.ca/gov/content/health/practitioner-professional-resources/bc-guidelines/chronic-kidney-disease>

Uric acid	207	140--360	umol/L	VRL
ALT	11	<36	U/L	VRL
Sodium	143	135--145	mmol/L	VRL
Potassium	4.4	3.5--5.0	mmol/L	VRL
Cholesterol	3.96	2.00--5.19	mmol/L	VRL
Triglyceride	0.54	<2.21	mmol/L	VRL
HDL-Cholesterol	1.22	>1.19	mmol/L	VRL
CHOL:HDL Ratio	3.25	<4.4		VRL
Non HDL-Cholesterol	2.74		mmol/L	VRL
Non HDL-cholesterol is calculated from total cholesterol and HDL-C and is not affected by the fasting status of the patient. The optimal non HDL-cholesterol level for intermediate and high risk individuals is <= 2.60 mmol/L. See Can J Cardiol 2013 vol 29 pgs 151 to 167.				
LDL Cholesterol	2.49	1.50--3.40	mmol/L	VRL
(SEE NEXT PAGE)				

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LDL Cholesterol

VRL

The optimal LDL cholesterol level for intermediate and high risk individuals is ≤ 2.00 mmol/L. If triglycerides are ≥ 1.50 mmol/L, consider monitoring of alternate lipid targets non HDL-cholesterol or apoB. For low risk individuals with LDL cholesterol ≥ 5.00 mmol/L, target reduction of LDL cholesterol ≥ 50 percent. See Can J Cardiol 2013 vol 29 pgs 151 to 167.

Hours p.c.

12.5

h pc

VRL

FERRITIN

112

15--247 ug/L

VRL

Adults >18 y: <15 ug/L: diagnostic of iron deficiency

15-30 ug/L: probable iron deficiency

 >30 ug/L: iron deficiency unlikely >100 ug/L: normal iron stores ≥ 600 ug/L: consider test for iron overload

See BC guideline for Iron Deficiency
Diagnosis and Management, 2019

C Reactive Protein

0.5 <5.0

mg/L

VRL

(SEE NEXT PAGE)

PATIENT'S NAME		LAB NUMBER	CHART NUMBER	INTERNAL USE ONLY		PG
MOSS NICOLE ASHLEY		409012011		7-SUM	Q1703	4
AGE:	35	DATE OF BIRTH:	30 NOV 1990	SEX:	F	PHN: 9836474537
PATIENT'S		POSTAL CODE:	V9L 1H9	REPORT DATE & TIME:	12 JAN 26	13:40
PHONE#:	(250) 510-2212					

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C Reactive Protein

VRL

Test method: Abbott Alinity hs-CRP, suitable for cardiovascular disease assessment and detection of active inflammation. CRP = >2.0 mg/L is a risk enhancing factor for cardiovascular disease, as defined in the Guidelines of the American Heart Association and the American College of Cardiology (JACC 2019: 74: e177).

TSH

1.25 0.32--5.04 mU/L

VRL

The FT4 and FT3 orders were cancelled as per BC Guidelines. However, the specimen will be stored for seven days.
Gestational reference intervals:
1st Trimester 0.10-3.50 mU/L
2nd Trimester 0.30-3.50 mU/L
3rd Trimester 0.45-4.00 mU/L

PROLACTIN

12.1 4.2--28.2 ug/L

VRL

Cortisol a.m.

296 140--535 nmol/L
WOKE AT 06:00

VRL

LH

3.1 IU/L
Follicular: 2.4 - 6.6 IU/L
Midcycle: 9.1 - 74.2 IU/L
Luteal: 2.4 - 6.6 IU/L
Postmenopausal: 11.3 - 58.0 IU/L

VRL

FSH

4.0 IU/L
(SEE NEXT PAGE)

VRL

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FSH

VRL

Follicular: <8.7 IU/L
Midcycle: 4.2-19.0 IU/L
Luteal: <8.7 IU/L
Postmenopausal: 16.0 -110.0 IU/L

PROGESTERONE

0.7 nmol/L

VRL

Follicular: < 1.7 nmol/L
Luteal: 2.2 - 60 nmol/L
Postmenopausal: < 0.7 nmol/L

The lower threshold to confirm
ovulation is 18 nmol/L.

ESTRADIOL

184 pmol/L

VRL

Follicular: 77 - 921 pmol/L
Midcycle: 139 - 2382 pmol/L
Luteal: 77 - 1145 pmol/L
Postmenopausal: <103 pmol/L

SHBG

30.5 20.0--180.0 nmol/L

VRL

When assessing testosterone status,
testosterone and SHBG should be tested on
the same specimen.
Reference interval is for non-pregnant
adult.

TESTOSTERONE (T)

0.5 <1.8 nmol/L

VRL

FREE T calculated
Method of Vermeulen

9 <30 pmol/L

VRL

BIOAVAILABLE T

0.2 <0.7 nmol/L
(SEE NEXT PAGE)

VRL

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PATIENT'S		POSTAL CODE:	V9L 1H9	REPORT DATE & TIME:	12 JAN 26	13:40
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BIOAVAILABLE T
Method of Vermeulen

Interpret BAT and cFT results with
caution in presence of significant
hypoalbuminemia.

VRL

hCG-Beta

<1

IU/L

VRL

Non Pregnant: <3
3rd Week: 6 - 71
4th Week: 10 - 750
5th Week: 217 - 7138
6th Week: 158 - 31795
7th Week: 3697 - 163563
8&9th Week: 32065 - 151410
10th Week: 46509 - 186977
12th Week: 27832 - 210612
14th Week: 13950 - 62530
15th Week: 12039 - 70971
16th Week: 9040 - 56451
17&18th Week: 8175 - 58176

RESULTS TO FOLLOW:

DHEAS

(END OF REPORT)